Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

COVID-19 Pre - Induction Checklist for Obstetric General Anaesthesia

Questions highlighted in bold to be read out loud during pre-oxygenation

Team

❑ Do we need to call for any more help?

Second Anaesthetist

1 - Bleep 063

2 - Bleep 550

ODP – Bleep 017

❑ Do we need to contact Consultant Anaesthetist on call?

Suggested involvement if:

- Cardiovascular instability
- Risk of major haemorrhage
- Obese (BMI >40)
- Predicted difficult airway

Confirm informed

❑ Are both the intubating anaesthetist and anaesthetic assistant wearing full PPE?

❑ Is the intubating anaesthetist double gloved ? (outer pair to be removed following intubation)

Patient

❑ Has the patient received Sodium Citrate ?

❑Is cardiovascular status optimal with left uterine displacement applied ?

❑ Is the patient in a head up / ramped position with left lateral tilt ?

Use :

Oxford HELP

or

Adjust the operating table to create the ramped position

❑ Is preoxygenation optimal?

- Preoxygenate using facemask & ensure a tight seal during preoxygenation to avoid aerosolisation
- Target ETO_2 > 90%
- **DO NOT** USE HIGH FLOW OXYGEN VIA NASAL CANNULA

Equipment

❑ Are necessary drugs drawn up and labelled ?

- Induction Agents
- Muscle Relaxant
- Vasopressor
- Atropine
- Syntocinon (available)
- Short acting opioid (Severe Pre-eclampsia)

❑ Is all monitoring on ?

- Capnography
- Pulse oximetry
- ECG & BP

❑ Is the anaesthetist happy with the equipment ?

- Videoscope e.g C-MAC
- 2 x Endotracheal Tubes
- Bougie
- Guedel airway
- 2nd gen. laryngeal mask
- Suction on & under pillow
- Ambu bag
- ETT clamp

Difficult Airway Plan

❑ What is the plan for a difficult Intubation ?

A – Videolaryngoscopy
e.g C-MAC, Pentax or Airtraq

B – 2nd generation laryngeal mask

C – Front of Neck Access (FONA) using scalpel-bougie-tube

❑ Is the relevant equipment for the above plan readily available?

including C-MAC video laryngoscope in position & switched on

❑ In the event of a failed intubation is the plan to wake up the patient or proceed with surgery under mask anaesthesia ?

Refer to Table 1 – 'Obstetric Failed Intubation guideline' overleaf

❑ Principles:

- **DO NOT ventilate until cuff inflation is confirmed**
- **Determine the position of the ETT using chest wall expansion & End Tidal CO2 ie. do not auscultate chest**
- **Clamp ETT before disconnection**
- **Check all breathing circuit connections to avoid inadvertent disconnection**
- **For extubation, please minimise the number of staff in the room due to the high risk of aerosolisation**

Is everyone happy to proceed ?

Proceed only if **Yes**

Adapted from the 4th National Audit Project of The Royal College of Anaesthetists

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